REVONIX SOLUTION - HEALTHCARE AI

PARASNATH AI

Intelligent Medical Voice Assistant & Digital Patient Documentation System

Full Project Blueprint

The complete plan for the system that turns one doctor-patient consultation into one AI-generated case paper, one doctor-approved prescription, and one permanent digital patient record - in Hindi, Marathi and English.

Organisation: Parasnath Speciality Clinic, Badlapur

Project type: AI-powered healthcare technology

Status: Phase 1 MVP built

12	10	3
Modules specified in this blueprint	Workflow steps from registration to delivery	Languages: Hindi, Marathi, English

> **The one rule that governs everything:** This is a documentation system, not an autonomous doctor. The AI never diagnoses, never invents a medicine or dose, and nothing is finalised or shared until a doctor approves it.

Prepared for Jyoti Bhanushali, Revonix Solution - 30 October 2026. This blueprint is the build reference for the live system and for any developer or AI tool rebuilding it.

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Status snapshot - 30 October 2026

> **Phase 1 MVP is built and working.** The web application (space: parsnath-ai) runs the full flow end to end: patient registration with a unique Patient ID, health questionnaire, consent-first consultation recording from a microphone or an uploaded audio file, transcript, structured case paper and prescription draft, doctor review and approval, PDF generation, and the record saved to the patient's profile. A knee-pain sample consultation sits unapproved in Approvals so the doctor flow can be tried immediately.

Item	State today
Transcription & AI extraction	Running on **clearly-marked demo (mock) adapters**. Pasting an OpenAI or Deepgram API key in Settings switches transcription to live. No key has been provided yet.
Theme & languages	Revonix deep-navy and purple theme. Interface in English, Hindi and Marathi.
WhatsApp / SMS delivery	Designed in (Phase 3). Blocked on the clinic's own Meta WhatsApp setup - see Open Items, page 15.
Production backend	This blueprint specifies the production target (FastAPI + PostgreSQL, Section 10). The current MVP is the working web artifact built to the same workflow and safety rules.
Patient data in testing	Synthetic data only. No real patient data is used in development or demos.

A note on module count: the brief's module table names 12 modules. Earlier summaries of the brief said 13 - that count is not supported by the table itself, so this blueprint specifies the 12 named modules and treats the dashboard and reports views as part of the Admin Dashboard and Patient CRM modules.

SECTION 1

Vision, objectives and design principles

PARASNATH AI removes the doctor's paperwork, not the doctor's judgement. It listens to a real consultation, drafts the case paper and prescription from what was actually said, and waits for the doctor to approve before anything becomes a record.

Problem it solves

Doctors at the clinic write case papers and prescriptions by hand after - or during - every consultation. That is slow, inconsistent between doctors, hard to search later, and leaves no central history when a patient returns months later or visits another Parasnath branch. PARASNATH AI replaces the writing, while keeping every clinical decision with the doctor.

Main objectives (from the approved brief)

- Automate doctor-patient consultation documentation end to end.
- Convert voice conversations in Hindi, Marathi and English - including mixed-language, code-switched speech - into structured medical records.
- Generate a digital case paper and a prescription draft from the actual consultation.
- Maintain a central patient record and medical history under one unique Patient ID.
- Deliver approved documents to patients digitally (WhatsApp, SMS link, portal or print).
- Connect patient records with the clinic CRM and support Excel / Google Sheets exports and management reports.
- Scale across multiple Parasnath branches without redesigning the core.

Design principles

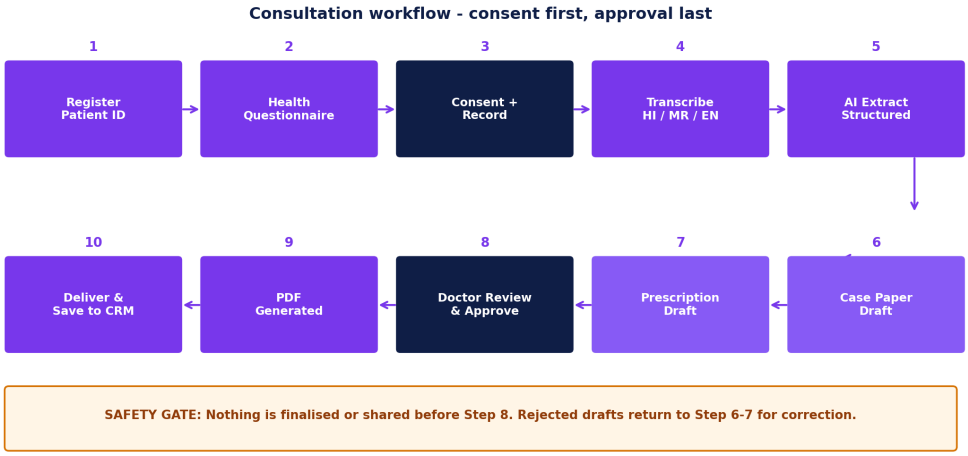
Principle	What it means in practice
Doctor-in-the-loop, always	Every AI output is a draft. Only a doctor's approval can finalise, generate a PDF or trigger delivery.
Nothing invented	The AI extracts only what was said or confirmed. Unclear medicine names, doses or findings are flagged, never guessed.
Patient voice vs doctor voice	Patient-reported symptoms are stored separately from doctor-confirmed findings, and suspected diagnoses separately from confirmed ones.
Consent before capture	Recording cannot start until patient consent for audio recording and processing is recorded.
Providers are swappable	Transcription, AI extraction, WhatsApp/SMS and storage sit behind adapters, so a provider can change without rebuilding modules.
Works without keys, honestly	Where no live API credential exists, a clearly-marked mock adapter keeps the whole flow testable - and is labelled as a mock everywhere it appears.

Success measures for Phase 1

These are targets to measure in the clinic pilot, not results: a consultation documented in minutes rather than written up afterwards; every approved consultation producing both PDFs without re-typing; zero documents shared before approval; and a returning patient found by Patient ID, name or mobile in seconds. Targets are confirmed with the clinic during the pilot - see Open Items.

SECTION 2

End-to-end workflow - the 10 steps



#	Step	What happens and what is produced
1	**Patient registration**	Register a new patient or search for an existing one. The system generates a unique Patient ID and stores name, age, gender and contact details.
2	**Digital health questionnaire**	Before the consultation: chief complaint, symptoms and duration, past history, current medicines, previous treatment, allergies and lifestyle. The questionnaire adapts to the complaint.
3	**Live voice recording**	With consent recorded, the doctor starts recording in the app. Speech is captured in Hindi, Marathi, English or mixed, with doctor and patient separated wherever technically possible.
4	**Transcription**	Audio becomes a readable transcript that recognises medical terminology. The doctor can review and correct it.
5	**AI extraction**	The transcript is organised into structured sections (detailed in Section 7), separating patient statements from doctor-confirmed findings.
6	**Digital case paper**	A 12-part case paper is generated: clinic and patient details, complaints, history, examination, assessment, treatment plan, advice, follow-up and the doctor's approval / signature area.
7	**Prescription draft**	Built only from doctor-stated or confirmed items: medicine, dosage, frequency, timing, duration, instructions and follow-up date. Anything unclear is flagged.
8	**Doctor review & approval**	The doctor edits the transcript, extraction, case paper and prescription, then approves or rejects. **This is the safety gate: nothing finalises before this step.**
9	**PDF generation**	On approval, a professional case paper PDF and prescription PDF are generated, saved to the patient's profile, and available to print or download.
10	**Delivery & CRM update**	Approved documents go to the patient via WhatsApp, an SMS link, the patient portal or a printed copy. The consultation, medicines and follow-up date update the patient's CRM record automatically.

Worked example - knee pain: a patient reports six months of knee pain, worse on stairs, in mixed Hindi and English. The system records it, extracts the six-month history, captures only the examination findings and diagnosis the doctor actually states, drafts the case paper and prescription, and holds both for approval before saving them against the Patient ID. The same flow applies to back, neck and shoulder pain, joint pain, arthritis and other clinic complaints. This example is the Phase 1 acceptance test (page 15) and is pre-loaded in the built MVP as an unapproved sample.

SECTION 3

Module specifications - modules 1 to 6

Each module has one job, defined inputs and outputs, and a named owner role. Modules talk through the API layer (Section 5); none of them calls an AI or messaging provider directly - that always goes through an adapter.

Module	Function & key behaviour	Inputs - Outputs
1. Patient Management Owner: Receptionist	Registration, unique Patient ID generation, patient profile. Duplicate check on mobile number before a new ID is issued. Search by ID, name or mobile.	In: registration details. Out: Patient ID, profile record.
2. Health Questionnaire Owner: Receptionist / Patient	Pre-consultation intake: complaint, symptoms and duration, history, current medicines, prior treatment, allergies, lifestyle. Question set adapts to the chief complaint.	In: patient answers. Out: structured intake attached to the visit.
3. AI Voice Assistant Owner: Doctor	Starts and stops consultation recording (microphone or uploaded audio), only after consent is recorded. Streams audio over WebSockets in the production build; shows recording state clearly at all times.	In: live / uploaded audio + consent flag. Out: stored audio, recording metadata.
4. Multilingual AI Owner: System	Speech recognition for Hindi, Marathi and English with in-sentence code-switching and medical terminology. Speaker separation (doctor / patient) wherever technically possible. Doctor can correct the transcript.	In: audio. Out: timestamped, speaker-labelled transcript.
5. Medical AI Extraction Owner: System	Converts the transcript into the structured schema in Section 7. Separates reported vs confirmed information; flags unclear or missing items instead of filling them in.	In: corrected transcript. Out: structured extraction (JSON) + flag list.
6. Digital Case Paper Owner: Doctor	Renders the extraction into the 12-part case paper format (clinic details, patient and ID, date and doctor, complaints, history of present illness, past history, examination, assessment, treatment plan, lifestyle and diet advice, follow-up, approval / signature area). Fully editable before approval.	In: extraction. Out: editable case paper draft.

Build status: All six modules are present in the Phase 1 MVP. Modules 4 and 5 run on mock adapters until live transcription and extraction keys are connected (Open Items, page 15).

SECTION 4

Module specifications - modules 7 to 12

Module	Function & key behaviour	Inputs - Outputs
7. AI Prescription Owner: Doctor	Prescription draft from doctor-confirmed items only: medicine name, dosage, frequency, timing, duration, instructions, treatment recommendations, follow-up date. Never invents a medicine or dose; unclear names are flagged for confirmation.	In: confirmed extraction items. Out: editable prescription draft + flags.
8. Doctor Approval Owner: Doctor	Review surface for transcript, extraction, case paper and prescription. Edit, correct terminology, confirm diagnosis, verify medicines, approve or reject. Approval is a recorded, auditable action - it is what unlocks PDF generation and delivery.	In: drafts. Out: approved documents or rejection with corrections.
9. PDF Generator Owner: System	Produces the professional case paper PDF and prescription PDF from approved content only, stores both in the patient profile, and supports print and download. Previous consultation documents remain on the record.	In: approved documents. Out: two PDFs, filed under Patient ID.
10. Patient CRM Owner: Admin / Doctor	The patient's complete history in one view: details, consultation history, case papers, prescriptions, medicine and treatment history, follow-up dates, treating doctor and next visit. Supports Excel / Google Sheets export and management reports.	In: every approved consultation. Out: patient timeline, exports.
11. Follow-up Management Owner: Receptionist	Tracks next appointments and follow-up dates set at approval, surfaces who is due, and (Phase 3) sends reminders through the delivery adapter. Reminder counters advance only after a message is confirmed sent.	In: follow-up dates. Out: due list, reminders.
12. Admin Dashboard Owner: Admin / Head Office	Clinic overview and control: total registered patients, today's consultations, total doctors, pending approvals, completed prescriptions, follow-up appointments, recent records, search by Patient ID / name / mobile, branch-wise reporting, doctor and branch management, settings and permissions.	In: all module data. Out: KPIs, reports, settings.

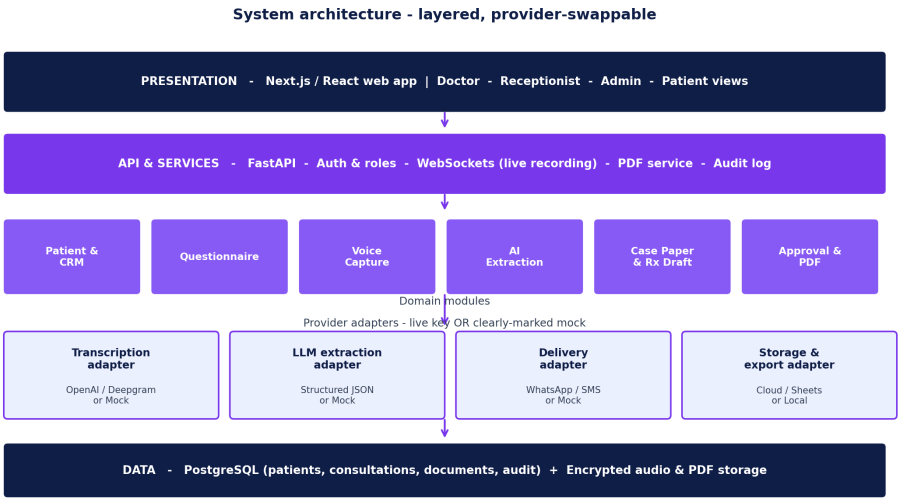
Dashboard KPI definitions

Every number on the dashboard is counted from live records, never typed in: **total patients** = registered profiles; **today's consultations** = consultations dated today; **pending approvals** = drafts awaiting a doctor decision (the queue that must never silently grow); **completed prescriptions** = approved prescriptions with a generated PDF; **follow-ups** = patients with a future follow-up date, with overdue ones shown first.

> **Build status:** Modules 7-10 and the dashboard KPIs are live in the Phase 1 MVP. Follow-up reminders (Module 11) and WhatsApp delivery activate in Phase 3, once the clinic's Meta WhatsApp setup exists.

SECTION 5

System architecture



How the layers work together

Layer	Responsibility and rules
Presentation	One responsive web app with role-specific views (doctor, receptionist, admin, patient). It holds no clinical logic - it displays drafts and captures edits and approvals.
API & services	FastAPI service layer: authentication, role checks on every request, WebSocket channel for live recording, the PDF service, and the audit log writer. Every state change (create, edit, approve, share) passes through here and is logged.
Domain modules	The 12 modules of Sections 3-4. They own the workflow rules - including the approval gate - and call providers only through adapters.
Provider adapters	One adapter per external capability: transcription, LLM extraction, delivery (WhatsApp / SMS), storage and export. Each adapter has a live implementation and a clearly-marked mock. Switching providers is a settings change, not a rebuild.
Data	PostgreSQL for structured records; encrypted object storage for audio files and PDFs. Audio and documents are linked to consultations, never stored loose.

Key architectural decisions

- **Approval is enforced in the service layer**, not just hidden in the UI - the PDF and delivery services refuse any document whose status is not "approved".
- **Drafts are versioned.** Doctor edits create a new version; the AI's original draft and the approved version are both retained for audit.
- **Branch-ready from day one.** Every patient, doctor and consultation carries a branch field, so Phase 4 multi-branch reporting needs no data migration.
- **Offline-tolerant recording.** If connectivity drops mid-consultation, audio is held locally on the device and uploaded when the connection returns; a consultation is never lost silently.

SECTION 6

Data model - one Patient ID is the spine

Every consultation, document and follow-up hangs off a single patient record. If it cannot be traced back to a Patient ID, it does not belong in the system.

Entity	Key fields	Relationships
Patient	Patient ID (unique, system-generated), name, age, gender, mobile, contact details, branch, registered date	Has many consultations, questionnaires, documents and follow-ups
Doctor	Doctor ID, name, qualifications, speciality, branch, login role	Conducts many consultations; approves documents
Branch	Branch ID, clinic name, address, logo, contact	Has many patients, doctors and consultations
Questionnaire	Visit link, chief complaint, symptoms, duration, history, current medicines, prior treatment, allergies, lifestyle	Belongs to one patient and one visit
Consultation	Consultation ID, Patient ID, Doctor ID, date and time, consent record (who, when), audio reference, status (draft / pending approval / approved / rejected)	Has one transcript, one extraction, one case paper, one prescription
Transcript	Consultation link, speaker-labelled segments, language tags, doctor-corrected version, provider used (live or mock)	Belongs to one consultation; versioned
Extraction	Structured JSON per the Section 7 schema, flag list (unclear / missing items), provider used	Belongs to one consultation; versioned
Case Paper	12-part document content, version, status, approved-by, approved-at	Belongs to one consultation; produces one PDF on approval
Prescription	Medicine lines (name, dosage, frequency, timing, duration, instructions), recommendations, follow-up date, status	Belongs to one consultation; produces one PDF on approval
Document (PDF)	File reference, type (case paper / prescription), consultation link, generated-at, delivery status	Belongs to one patient and one consultation
Follow-up	Patient link, due date, reason, reminder count, last reminder confirmed-at	Belongs to one patient; created at approval
Audit Log	Actor, role, action, entity, timestamp, before / after reference	Records every create, edit, approval, export and share
User & Role	User ID, name, role (Admin / Doctor / Receptionist / Patient), branch scope, active status	Controls access per Section 8

> **Data rules:** Patient IDs are never reused or deleted. Medical documents are never hard-deleted - corrections create versions. Development and demos use synthetic patients only.

SECTION 7

AI and voice pipeline

The pipeline has four stages - capture, transcribe, extract, draft - and one hard rule between every stage: uncertainty is flagged for the doctor, never smoothed over.

Stage by stage

Stage	What happens, and what can go wrong
1. Capture	Browser microphone recording in the app, or an uploaded audio file. Consent is checked before recording can start. Recording state is always visible; pausing and resuming keep one continuous consultation file.
2. Transcription	Speech-to-text via the transcription adapter (OpenAI or Deepgram when a key is connected; mock otherwise). Must handle Hindi, Marathi and English, code-switching inside a sentence, and medical terms. Speaker diarisation labels doctor and patient segments wherever technically possible.
3. Extraction	An AI language model reads the (doctor-correctable) transcript and returns structured JSON in the fixed schema below - no free-form summaries. Every field must trace to something said in the consultation.
4. Drafting	The extraction fills the case paper and prescription templates. Fields with no source in the transcript stay empty and appear on the flag list - they are never completed from general medical knowledge.

Extraction schema - what the AI must return

- Symptoms as reported by the patient
- Duration and severity of symptoms
- Relevant medical history
- Doctor's clinical observations
- Doctor's assessment - suspected vs confirmed, kept separate
- Treatment recommendations
- Medicines mentioned, with dosage and frequency only if explicitly stated
- Lifestyle and dietary instructions
- Follow-up recommendation and date
- Flag list: unclear names, missing dose / duration, low-confidence segments

The validation that comes before everything else

> **Mixed-language accuracy is the project's top technical risk.** The brief requires the technology choices to be validated through a prototype, specifically on Hindi-Marathi-English mixed medical conversation. Before live rollout: record a set of synthetic test consultations (including the knee-pain example), run them through each candidate transcription provider, and compare medicine names, doses and findings against the known script. The provider that survives that test gets the key - not before.

Medicine catalogue (Phase 2): extracted medicine names are matched against a clinic-approved medicine catalogue. A name that matches nothing is flagged, never auto-corrected to a similar-sounding medicine - in a prescription, a confident wrong guess is worse than an honest blank.

SECTION 8

Safety, privacy and user roles

The ten compulsory safety rules (from the brief, unedited in intent)

1. The AI must not independently diagnose patients.
2. The AI must not invent symptoms, clinical findings, medicines or dosages.
3. Patient-reported information is kept separate from the doctor's observations.
4. Suspected diagnoses are distinguished from confirmed diagnoses.
5. Unclear or missing information is flagged for doctor review.
6. All AI-generated case papers and prescriptions stay in draft status until doctor approval.
7. Unapproved medical documents are never shared - with anyone, on any channel.
8. Appropriate patient consent is obtained for audio recording and processing, before recording.
9. Patient information is protected with encryption, access controls, audit logs and secure storage.
10. The system follows applicable Indian data-protection and healthcare requirements, and uses synthetic patient information during development and testing.

Who can do what

Capability	Admin / Head Office	Doctor	Receptionist	Patient
Manage branches, doctors, settings, permissions	Yes	No	No	No
Register patients, issue Patient IDs, manage appointments	Yes	No	Yes	No
Collect health questionnaire	Yes	Yes	Yes	Own only
Start / stop consultation recording	No	Yes	No	No
Review, edit and approve case papers & prescriptions	No	Yes	No	No
View patient history	Per permissions, all branches	Own patients	Permitted records only	Own documents only
Receive approved documents	-	-	-	Yes
View reports & analytics	Yes, incl. branch-wise	Own summary	No	No

Privacy in practice: role checks run on the server for every request, not just in the interface. Audio and PDFs are encrypted in storage. Every view, edit, approval, export and share writes an audit log entry. A patient only ever receives documents that a doctor has approved.

SECTION 9

Integrations and API contracts

Every external service sits behind an adapter with the same contract: a live implementation when credentials exist, and a clearly-marked mock when they do not. Modules never know which one is running - but the interface always shows which one is.

Integration	Contract	Live provider	Mock behaviour today
Transcription	Audio in -> speaker-labelled, timestamped transcript out, with language tags	OpenAI speech-to-text or Deepgram (key pasted in Settings)	Returns a labelled sample transcript so the full flow can be tested
AI extraction	Transcript in -> structured JSON in the Section 7 schema + flag list out	AI language model with structured JSON outputs	Returns a structured sample extraction (the knee-pain case)
WhatsApp delivery	Approved document + patient mobile in -> delivery confirmation out	Official Meta WhatsApp API, clinic's own app and number (Phase 3)	Marks delivery as "simulated" - nothing is sent
SMS delivery	Patient mobile + secure document link in -> send confirmation out	SMS provider, to be selected	Simulated send, logged as mock
Export	Date range / branch filter in -> Excel or Google Sheets file out	Sheet generation from CRM data	Same export, from sample data in the MVP
Storage	File + consultation link in -> encrypted stored reference out	Cloud object storage	Local artifact storage in the MVP

Core API surface (production build)

Endpoint group	Purpose
`POST /patients` `GET /patients?search=`	Register a patient (returns Patient ID); search by ID, name or mobile
`POST /consultations` `POST /consultations/{id}/audio`	Open a consultation (consent required in the payload); attach or stream audio
`POST /consultations/{id}/transcribe` `/extract`	Run the transcription and extraction adapters; results stored as drafts
`PUT /consultations/{id}/case-paper` `/prescription`	Doctor edits; each save creates a new version
`POST /consultations/{id}/approve`	The gate: validates the approver is a doctor, locks the approved version, triggers PDF generation
`GET /patients/{id}/history` `GET /dashboard/kpis`	Full patient timeline for the CRM; live-counted dashboard numbers
`POST /documents/{id}/deliver`	Send an approved document via WhatsApp / SMS; refuses non-approved documents

All endpoints require authentication and enforce the role matrix on page 10. Provider keys live in Settings as secrets: never shown after entry, never in exports or logs.

SECTION 10

Technology stack, deployment and quality requirements

Recommended stack (from the brief, adopted)

Frontend

- Next.js, React, TypeScript
- Tailwind CSS, Shadcn UI
- Revonix deep-navy and purple theme, English / Hindi / Marathi interface

Backend

- Python, FastAPI
- SQLAlchemy, Pydantic (schema validation on every extraction)
- WebSockets for live recording

Data & AI

- PostgreSQL
- OpenAI speech-to-text / real-time transcription APIs, or Deepgram
- AI language model with structured JSON outputs
- Medical terminology and medicine catalogue (Phase 2)
- Speaker identification / diarisation

Integrations

- PDF generation, WhatsApp API, SMS
- Excel / Google Sheets export, cloud storage
- Authentication and role-based access

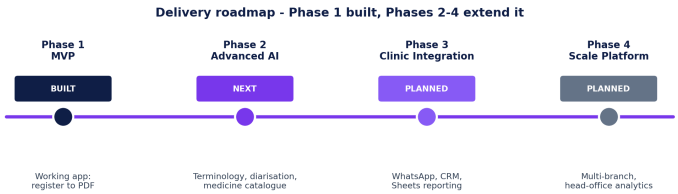
> **Honest note on the current MVP:** the Phase 1 application already delivered is a working web artifact that implements this workflow, data shape and safety model. The stack above is the production target for hardening and scale (Phases 2-3); moving to it does not change the workflow, the screens or the safety rules in this blueprint.

Non-functional requirements

Requirement	Standard
Responsive use	Full workflow usable on a phone, tablet and desktop - consultations are recorded at the desk or bedside, not only in an office.
Draft safety	No approval, no PDF, no delivery - enforced in the service layer and covered by tests (page 15).
Auditability	Every create, edit, approval, export and share is logged with actor, role and time.
Data protection	Encryption in storage and in transit, role-based access, secrets held outside the codebase, synthetic data in all non-production environments.
Recovery	Regular database and file backups; a lost connection during recording never loses the consultation audio.
Provider independence	Any adapter can be replaced without touching module code; the system runs end to end on mocks for demos and training.

SECTION 11

Roadmap - Phases 1 to 4



Phase	Status	Scope
Phase 1 - Working MVP	**BUILT**	Patient registration and unique Patient ID; health questionnaire; audio recording and transcription; multilingual support; AI extraction; digital case paper; prescription draft; doctor approval; PDF generation; patient record storage. Delivered as the parsnath-ai web app with demo adapters and the knee-pain sample waiting in Approvals.
Phase 2 - Advanced AI	Next	Live transcription and extraction keys connected and validated on synthetic mixed-language tests; improved medical terminology recognition; better speaker identification; context-aware extraction; dynamic questionnaires; medicine catalogue matching; follow-up automation groundwork.
Phase 3 - Clinic Management Integration	Planned	WhatsApp delivery of approved documents (clinic's own Meta setup); CRM integration; Excel / Google Sheets reporting; full admin and doctor dashboards on the production backend; SMS delivery.
Phase 4 - Scalable Healthcare Platform	Planned	Centralised head-office dashboard; branch-wise analytics; advanced patient history; integration with other clinic management systems; expandable architecture for additional Parasnath branches.

What "done" means at each phase

- **Phase 1** is done when the acceptance criteria on page 15 pass in the built app.
- **Phase 2** is done when live transcription beats the agreed accuracy bar on the synthetic test set - especially medicine names and doses - with flags catching the rest.
- **Phase 3** is done when a patient actually receives an approved prescription on WhatsApp from the clinic's number, and the delivery is confirmed in the record.
- **Phase 4** is done when a second branch runs on the same system and head office sees branch-wise reports without manual consolidation.

SECTION 12

Effort estimates and risks

Estimates below are planning ranges for the remaining work after the built Phase 1 MVP, assuming one builder with AI assistance and timely clinic inputs. They are planning aids, not commitments - the open items on page 15 move them more than the coding does.

Work package	Estimate	Depends on
Phase 2: connect live transcription + extraction, synthetic test set, accuracy validation	1-2 weeks	API key (OpenAI or Deepgram); clinic time to review test outputs
Phase 2: medicine catalogue + dynamic questionnaires	1-2 weeks	Clinic-approved medicine list from the doctors
Phase 3: production backend hardening (FastAPI + PostgreSQL, auth, audit, backups)	2-3 weeks	Hosting decision and accounts
Phase 3: WhatsApp delivery + follow-up reminders	1-2 weeks	Clinic's Meta app, WhatsApp number and template approvals
Phase 3: Sheets / Excel reporting pack	3-5 days	Report formats the clinic wants
Phase 4: multi-branch + head-office analytics	2-4 weeks	Second branch's doctors, data and rollout plan
Clinic pilot, training and fixes (runs alongside Phases 2-3)	1-2 weeks elapsed	Doctor and receptionist availability for feedback

Risks and mitigations

Risk	Impact	Mitigation
Mixed Hindi-Marathi-English transcription mis-hears a medicine name or dose	High	Prototype validation before rollout (page 9); medicine catalogue matching; every uncertain item flagged; doctor approval gate means an error must survive a human check to reach a patient
A draft is shared before approval (process or software slip)	High	Gate enforced in the service layer, not just the UI; delivery API refuses non-approved documents; audit log on every share
Patient privacy breach or consent gap	High	Consent recorded before capture; encryption, role-based access, audit logs; synthetic data outside production; consent wording approved by the clinic (Open Item 5)
Live features stall waiting for API keys / Meta setup	Medium	Mock adapters keep demos, training and the full workflow running; open items list names exactly who provides what
Doctors find review slower than writing	Medium	Pilot with real consultations early; measure review time; improve extraction prompts and templates from doctor corrections
Scope grows before Phase 1 is accepted	Medium	Acceptance criteria fixed on page 15; new ideas go to Phases 2-4, not into the MVP

SECTION 13

Open items, decisions needed and acceptance criteria

Open items - what is needed, and from whom

#	Item	Needed from	Unlocks
1	Live transcription / AI API key - OpenAI or Deepgram - pasted into the app's Settings (never by chat or email)	Jyoti / clinic account owner	Real transcription and extraction (Phase 2)
2	Meta WhatsApp setup: clinic's Meta app, WhatsApp Business number, and message template approval	Clinic, with guided setup support	WhatsApp delivery and reminders (Phase 3)
3	Real clinic data to replace samples: doctors list, timings, branches, logo and letterhead details	Jyoti / clinic	Clinic-ready PDFs and dashboards
4	Clinic-approved medicine catalogue (names and standard doses actually prescribed)	Doctors	Catalogue matching and safer drafts (Phase 2)
5	Patient consent wording for recording and processing, approved for clinic use	Clinic	Live recording with real patients
6	Hosting and domain decision for the production system	Jyoti / clinic	Production deployment (Phase 3)
7	Pilot plan: which doctor(s), which days, and how feedback is collected	Clinic	Accuracy measurement and Phase 2 sign-off

Phase 1 acceptance criteria - the MVP passes when all of these hold

1. A new patient can be registered and receives a unique Patient ID; searching by that ID, name or mobile finds them.
2. The pre-loaded knee-pain consultation can be opened in Approvals, edited, and approved - and only then does its PDF exist.
3. Before approval, the same consultation produces no PDF and cannot be delivered on any channel.
4. Recording cannot start without the consent box ticked; audio upload works as an alternative to the microphone.
5. The case paper shows all 12 sections; the prescription shows medicine, dosage, frequency, timing, duration, instructions and follow-up date - unclear items flagged, not filled in.
6. After approval, the consultation, its PDFs, medicines and follow-up date appear in the patient's history and in the dashboard counts.
7. Wherever the demo adapters are active, the screen says so - nothing mock is presented as live.
8. The full flow works in the Revonix theme in English, Hindi and Marathi, on phone and desktop.

> **Next step:** Jyoti reviews this blueprint and the built MVP together. Approving the blueprint freezes Phase 1 scope; the first build action after that is Open Item 1 - connecting a live transcription key and running the synthetic accuracy test on page 9.

PARASNATH AI - one consultation, one AI-generated case paper, one approved prescription, one permanent digital patient record. Blueprint prepared 30 October 2026 for Parasnath Speciality Clinic, Badlapur - Revonix Solution.